

EBOLA VIRUS DISEASE

Democratic Republic of the Congo

External Situation Report 02

Date of issue: 23 September 2025

Data as reported by: 21 September 2025

Situation update	Cases	Deaths	CFR
	 57	 35	61.4%

The Ebola virus disease outbreak officially declared by the Ministry of Health of the Democratic Republic of the Congo on 4 September 2025 continues to unfold. Since our last update ([Situation Report # 1](#)) issued on 15 September 2025, a total of 11 new confirmed cases have been reported from Bulape Health Zone in Kasai Province. The outbreak remains geographically confined to six affected health areas (Bambalaie, Bulape, Bulape Communautaire, Dikolo, Ingongo and Mpianga) within Bulape Health Zone. Ten deceased patients who were not sampled and tested were also retrospectively reclassified as probable cases based on epidemiological links to confirmed cases.

As of 21 September 2025, a total of 57 cases (47 confirmed and 10 probable cases), including 35 deaths (25 confirmed and 10 probable) (overall case fatality ratio 61.4%) have been reported from Bulape Health Zone, Kasai Province, Democratic Republic of the Congo. There are five confirmed cases among health workers.

Cases range from zero to 65 years old, with age group 0 – 9 years the most affected, accounting for 23% of the total cases. Females account for 61% of cases. Case fatality ratio among females is 56% compared to 73% among males. Children, housekeepers and farmers are the most affected groups.

The outbreak shows a decreasing trend of cases in the recent week, nevertheless the attention remains high, and response activities are ongoing in all affected health areas including early case detection, isolation, case management, contact tracing, vaccination as well as risk communication and community engagement.

As of 21 September 2025, a total of 1 180 contacts has been identified and are being followed up. Of these, 94 completed 21 days of monitoring while the rest remain under follow-up. Of 26 cases admitted to the Ebola Treatment Centre since the onset of the outbreak, two (2) were successfully treated and discharged following recovery on 16 September 2025, five died and 19 are currently in treatment as of 21 September 2025.

Figure 1: Epidemic curve of confirmed and probable Ebola virus disease (EVD) cases in Bulape Health Zone, Kasai province, Democratic Republic of the Congo, 16 August – 21 September 2025

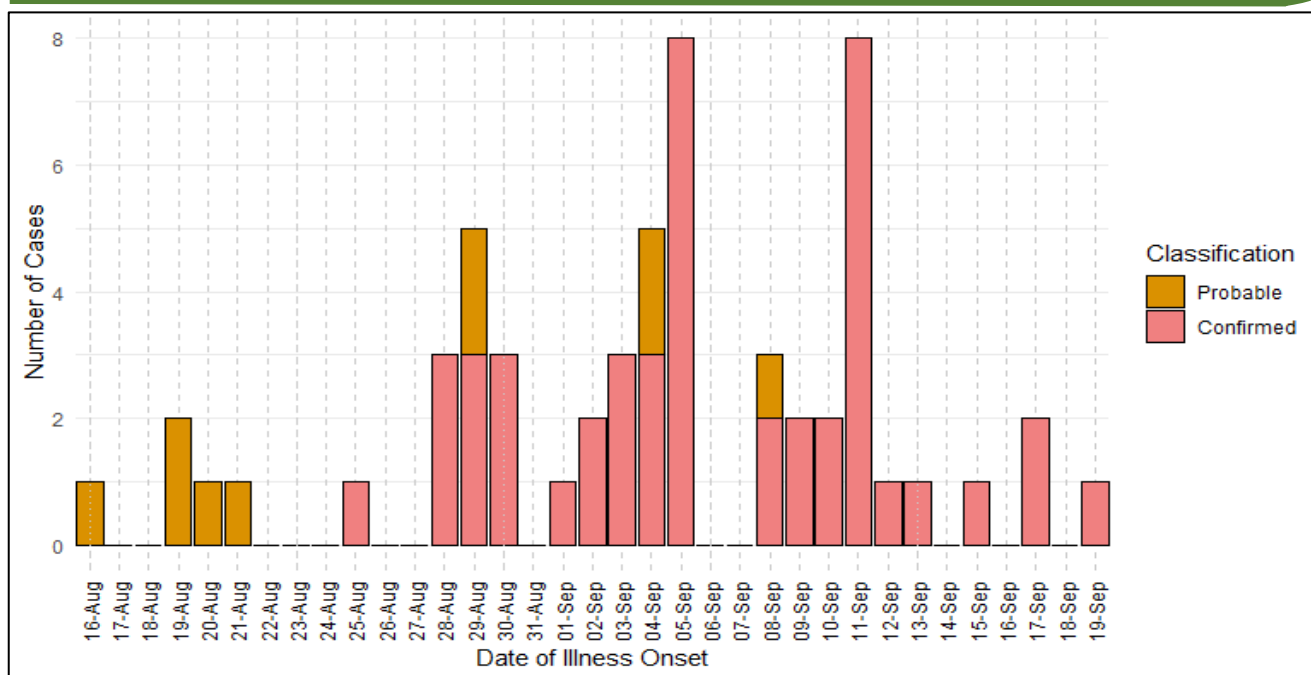
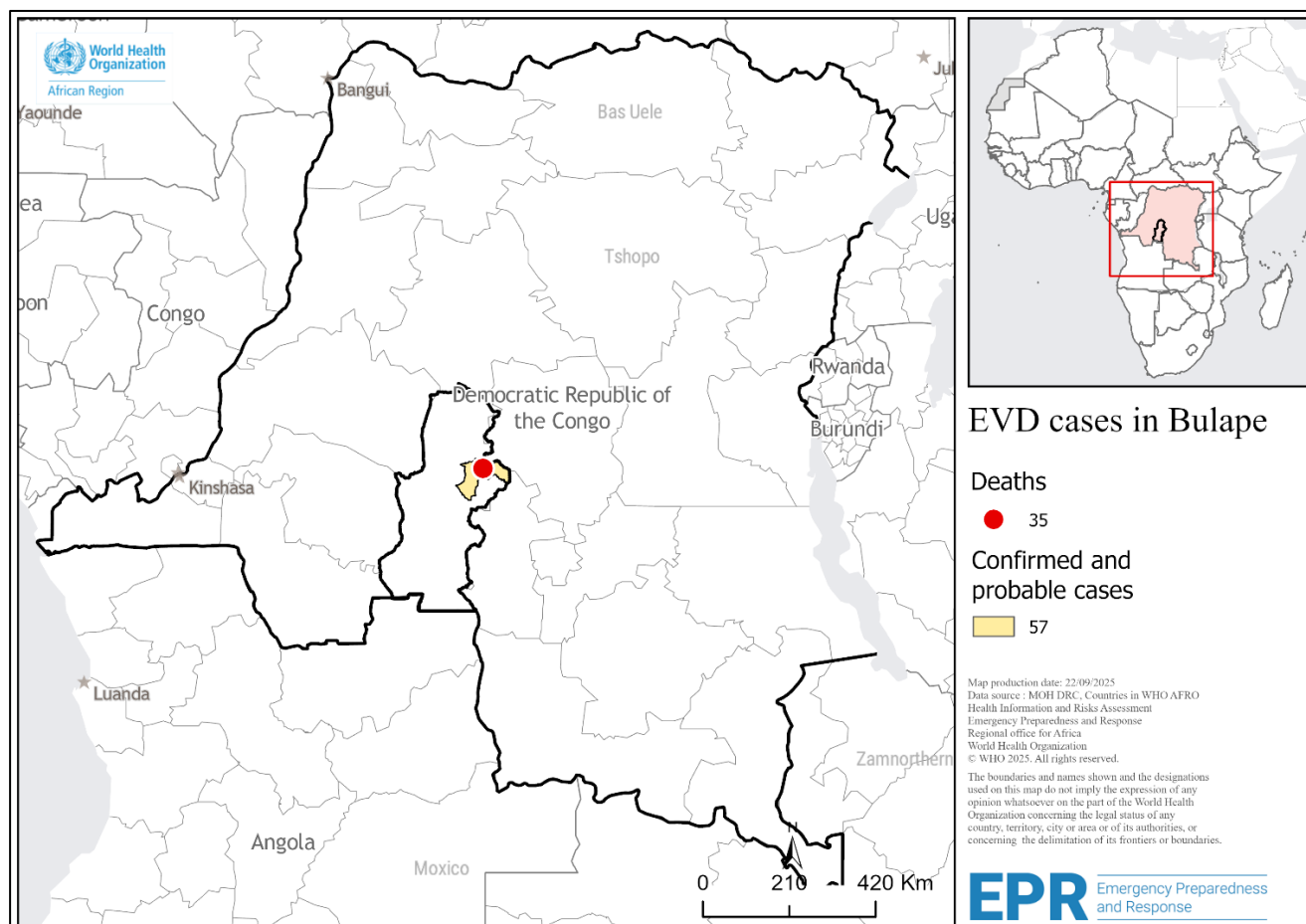


Figure 2: Geographic distribution of cases and deaths of Ebola virus disease, Kasai province, Democratic Republic of the Congo, as of 21 September 2025



2. Public Health Response

Coordination

- The Ministry of Health, through the National Public Health Institute (INSP) and the National Public Health Emergency Operations Centre (COUSP), continues to coordinate the overall response to the outbreak, with technical support from WHO and health partners.
- A dedicated coordination platform through an integrated Incident Management Team has been established in Bulape Health Zone, bringing together the Ministry of Health response team, WHO, and health partners to synchronize activities, align priorities, and review resource allocations. This has been marked by strong multi-partner collaboration, with joint missions and teams deployment for surveillance, case management, and community engagement, among others. Daily coordination meetings are being held to plan and guide response activities.
- As of 21 September 2025, WHO had deployed 64 experts, of which 78% (n=50) are nationals, including 21 from the Africa Volunteer Health Corps Strengthening and Utilizing Response Groups for Emergencies (AVOHC SURGE). Of the total experts deployed, 51 are stationed in Bulape Health Zone, epicentre of the outbreak.
- Other health partners supporting response efforts in Bulape include Africa CDC, International Organization for Migration (IOM), UNICEF, MSF, the Alliance for International Medical Action (ALIMA), the International Medical Corps (IMC), the Congolese Red Cross, and US Centers for Disease Control and Prevention. Others providing support include Family Health International 360 (FHI360), Program for Appropriate Technology in Health (PATH), Village Reach, World Food Program (WFP), UNFPA, the GAVI Alliance, Research Triangle Institute (RTI), FCDO, the UK Foreign, Commonwealth & Development Office, DG ECHO and the World Bank.
- A regional strategic response plan has been developed to guide coordinated efforts across affected and at-risk areas, focusing on surveillance, diagnostics, vaccination, infection, prevention and control (IPC), and community engagement.
- WHO has launched an appeal of US\$ 21 million to support the Democratic Republic of the Congo to scale up response operations to interrupt all transmission chains, control the outbreak, and mitigate the impact on human health. So far, US\$2 million has been released from WHO's Contingency Fund for Emergencies to finance immediate response activities.

Surveillance and Laboratory

- Surveillance activities continued to be scaled up in Bulape Health Zone and proximal areas. Two hotlines have been set up for reporting alerts and suspected cases. All alerts and reported cases are being investigated.
- Community health workers are also being trained to strengthen community-based surveillance of Ebola virus disease. So far, a total of 21 community health workers have been trained in the affected areas to report alerts and suspected cases using simplified case definitions. Local volunteers from the Congolese Red Cross

have also been engaged to strengthen community-based surveillance, including reporting of deaths in the community.

- Mortality surveillance has been instituted in health facilities to rule out any potential Ebola death. Reviews of health facility registers to identify any missed cases have commenced.
- Contact tracing activities continue, with 1 180 contacts identified and followed as of 21 September 2025. Of these, 94 contacts have completed 21 days of follow-up. Among the remaining contacts, 929 were seen on 21 September 2025.
- A total of 177 samples have been tested since the onset of the outbreak. Of these, 47 were confirmed positive by RT-PCR testing. A mobile laboratory deployed to Bulape Health Zone since 19 September 2025, continues to facilitate the rapid turnaround of the testing of samples.
- Laboratory supplies are being enhanced with additional 400 GeneXpert cartridges delivered in the field. A total of five (5) Uninterruptible Power Supply (UPS) to ensure backup power to laboratory equipment, two biosafety cabinets, and various consumables have been dispatched to the testing facilities.
- The Ebola virus disease diagnostic strategy has been disseminated to all testing laboratories.
- Genomic sequencing of samples is also being strengthened. Three biologists have been deployed to set up genomic sequencing at a laboratory in the neighbouring Mweka Health Zone, which is 35 kilometres from Bulape Health Zone. Sequencing activities are expected to begin on 24 September 2025 at the laboratory in Mweka Health Zone.

Case Management

- Bed capacity at the Bulape Ebola Treatment Centre (ETC) has been expanded from 18 beds to 34. As of 21 September 2025, 19 confirmed and 15 suspected cases were in admission, giving a bed occupancy rate of 100.0%.
- To date, 25 patients have received treatment with the monoclonal antibody (mAb114), also known by its generic name ansuvimab-zykl (Ebanga). Furthermore, supportive and nutritional care are being provided to patients in the ETC. Five of these patients died while one recovered.
- A surge team of health care workers has been deployed by the MoH and Institut National de Santé Publique (INSP) to support clinical care, including trained specialists from the pool of rapid responders and the national emergency medical team.
- ALIMA, MSF, and INSP are actively supporting case management activities.
- Additional staff are being deployed to relieve existing staff at Bulape General Hospital who were listed as contacts so as to ensure continuity of care to admitted patients.

Vaccination

- Vaccination with rVSVΔG-ZEBOV-GP (Everbo) Ebola vaccine continues in the affected areas. As of 21 September 2025, a total of 1 740 individuals have been vaccinated in Bulape, Bulambae and Mweka Health Zones over a period of nine days since the commencement of vaccination activities.
- The balance stock of 2 410 doses out of the 4 150 doses already deployed to the affected area will be boosted with additional supplies in the coming days.
- A ring vaccination approach targeting contacts and potential-contacts, healthcare and frontline workers (HCW/FLW) at high risk is being implemented. This will be complemented by a targeted geographic approach in all hotspots where cases have been confirmed with the additional doses received.
- Installation of the Ultra Cold Chain equipment is ongoing in Tshikapa, Kananga and Mweka to accommodate the vaccines in the field.

Infection prevention and control (IPC)

- Efforts to enhance IPC practices continue. Efforts have focused on Bambalaie Health Area, where households and the health centre linked to confirmed cases were decontaminated, and an IPC assessment revealed critical gaps. Support included monitoring and coaching of health staff on injection safety, waste management, and sterilization, alongside the establishment of an isolation unit and provision of PPE.
- To reinforce IPC measures, more than 77 health workers in Bulape Health Zone have been trained on basic IPC practices, including safe use of PPE and proper waste management. Relevant health care providers are also being trained on the job for screening and triaging of patients using case definitions, standard and transmission-based precautions.
- Additionally, community engagement sessions are being held with local leaders, women, and youths to reinforce knowledge on Ebola transmission and the importance of preventive practices such as systematic handwashing.
- Three Safe and Dignified Burial (SDB) teams with 24 members are actively conducting burials for both confirmed and suspected cases, completing most within 24 hours of notification. As of 21 September 2025, a total of 22 safe and dignified burials of confirmed cases have been conducted by the burial teams in line with established protocols.
- Eighteen Red Cross members have been trained to take over SDB responsibilities, supported by adequate supplies (including a recent UNICEF delivery) and a three-month forecast.

Prevention of Sexual Exploitation, Abuse and Harassment (PRSEAH)

- Prevention of sexual exploitation, abuse, and harassment (PRSEAH) continues to be mainstreamed in the response activities. Activities include briefing of all responders, community sensitization sessions, and community risk analysis.

- A total of 285 individuals across three local churches in Bulape Health Zone were orientated. Additionally, 129 individuals involved in the response as well as community members have been briefed on reporting mechanisms.
- PRSEAH sensitizers have been identified in collaboration with the Chief Medical Officer of the Bulape Health Zone.
- Posters were displayed at the head nurses' offices in the health centres and the central health zone office to inform staff and visitors about PRSEAH principles and reporting mechanisms.

Risk communication and community engagement (RCCE)

- Risk communication and community engagement activities are being scaled up in Bulape Health Zone and proximal areas through household visits, community briefings, and sensitization sessions. Over 10 000 people have been directly reached through household visits.
- Community leaders and community health workers have been engaged to enhance awareness on Ebola virus disease signs and symptoms, transmission, and preventive measures.
- Leaflets have been distributed, mapping of churches and local leaders completed, and targeted interventions carried out to address refusals and rumours, including cases of resistance to isolation and transfer to treatment centres.
- RCCE activities are being mainstreamed into contact tracing and vaccination efforts in the affected communities.
- A rapid assessment was conducted and finalized on 9 September 2025, providing baseline information on knowledge, attitude, and practices in the community. The assessment found low community knowledge about the disease, defiance towards health system, and high-risk behaviours, among others. The result of this assessment is being used to tailor RCCE interventions.
- To continue to expand and intensify RCCE activities, community health volunteers (RECOs) in the affected health areas are being trained. So far, 65 RECOs have been trained to create awareness and engage community members.

Operations Support and Logistics

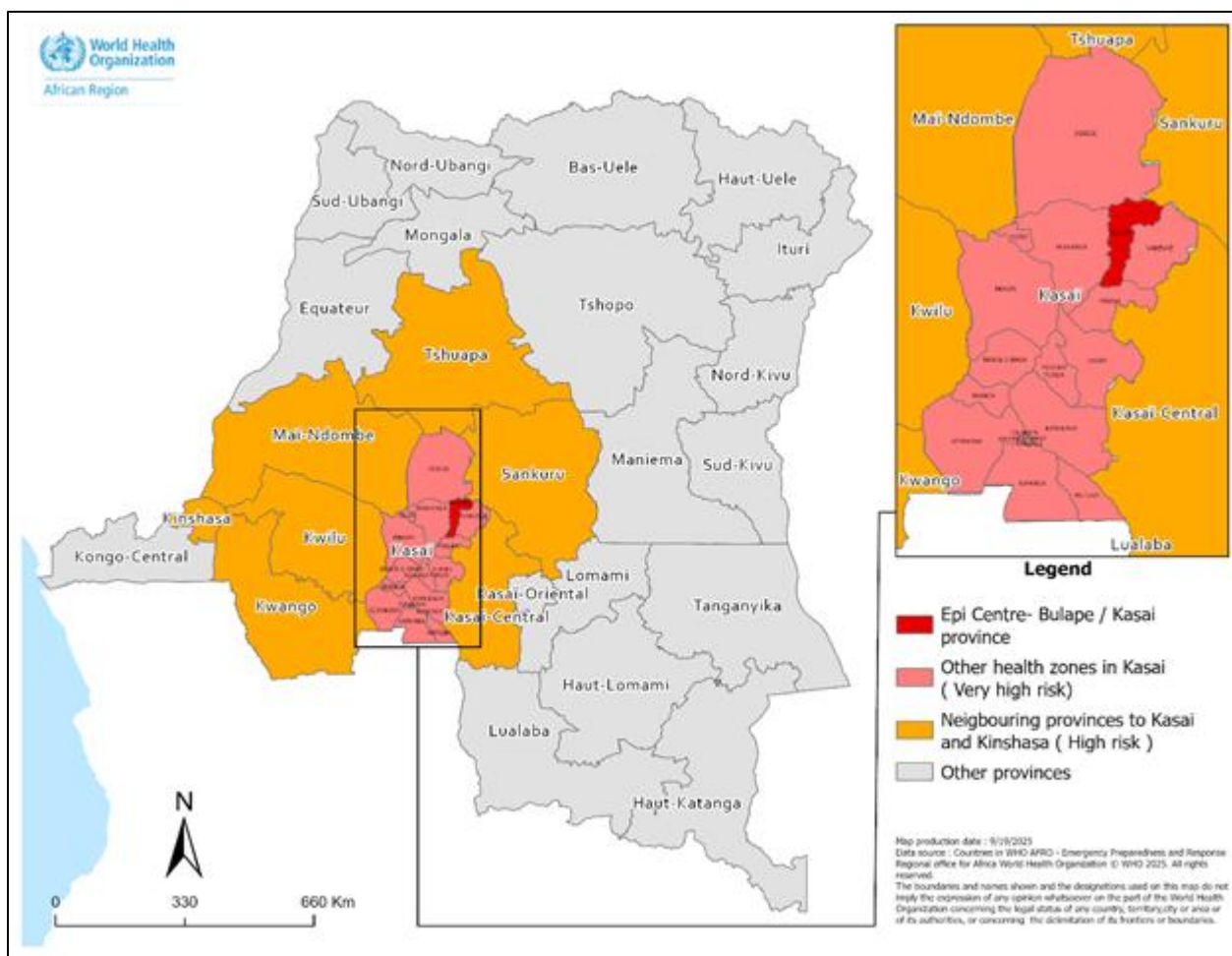
- WHO and partners have rapidly scaled-up operational and logistical support in synergy with the efforts of the government of the Democratic Republic of the Congo to respond to the outbreak.
- To fast-track delivery of supplies and personnel, a collaboration between WHO, WFP and the United Nations Organization Stabilization Mission in the Democratic Republic of the Congo (MONUSCO) has established a temporary airbridge for two weeks using a helicopter from Kananga to Bulape and Mweka.
- An additional 2.5 tons of essential medical supplies and equipment were delivered by WHO to the Bulape Health Zone to support response activities. This is in addition to over 14 tons of essential medical supplies and equipment delivered in the past week with air support from the WFP and MONUSCO to access the affected health zone. The supplies which will cover an additional 120 patients include personal protective equipment (PPE), testing kits, tents, and materials to set up a treatment centre.

- Other logistics provided include epi-shuttle, a specialized medical isolation and transport system designed for safe transfer of patients with highly infectious patients, 22 KVA generator to provide electricity for the ETC, six motorbikes for surveillance activities, mattresses and food items for patients.

Operational Readiness

- Eight of the nine neighbouring countries have completed readiness assessments, with an overall regional readiness score of 57%. The results are: Uganda (80%), Rwanda (81%), Burundi (68%), United Republic of Tanzania (65%), Zanzibar (64%), Central African Republic (42%), Republic of Congo (37%), South Sudan (41%), and Zambia (37%).
- In DRC, the neighbouring high-risk provinces are being supported for readiness assessment and contingency planning. Readiness and preparedness plan for nearby Sankuru Province has been completed.
- Readiness training have been planned for five at-risk health zones (Mweka, Mushenge, Luebo, Dekese, Kakenge) on surveillance, IPC, risk communication, and community engagement.

Figure 3: Ebola virus disease risk assessment for health zones and provinces close to the outbreak epicentre as of 21 September 2025, Democratic Republic of the Congo



Domestic and international traffic-related measures and cross-border health

No international traffic-related measures are currently warranted. Health authorities are reinforcing surveillance at border crossings, including health screening, proactive risk communication with travellers at points of entry/points of crossing (PoEs/PoCs), sensitization for PoE staff to detect, report and manage suspected cases, integration of border communities in affected areas into early warning systems and the national surveillance network, as well as coordination with IOM. WHO continues to monitor cross-border risks and provide technical support to mitigate the risk of cross-border spread.

Situation Interpretation

The current Ebola virus disease outbreak in Bulape Health Zone, Kasai Province, represents a serious public health threat. The outbreak is likely to have originated from a new zoonotic spillover and has already resulted in sustained human-to-human transmission, including infections and deaths among health workers, which heightens the risk of further nosocomial amplification. The case fatality ratio of 61.4% underscores the high impact on human health. The impact on children, women, and frontline groups such as farmers and caregivers highlights both the community-based nature of exposure and the potential for household and occupational transmission chains.

The rapid response mounted by national authorities and partners must be commended. However, with the onset of the rainy season, which will intensify in the months ahead, worsening road conditions are expected to challenge response efforts, making it harder to reach affected communities and deliver critical supplies. The next two to three weeks will be critical for containing transmission in Bulape Health Zone and reducing the risk of further spread. Government and partners need to maintain the momentum of the current response by strengthening surveillance, reinforcing IPC standards, expanding vaccination, and sustaining community engagement. Central to this will be the availability of funding to scale up these interventions.

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Correspondence on this publication may be directed to:

Dr Patrick Otim RAMADAN
Program Area Manager
Epidemics and Humanitarian Health Response (EMR)
WHO Emergency Preparedness and Response
WHO Regional Office for Africa
P O Box. 06 Cité du Djoué, Brazzaville, Congo

Dr Etien Koua
Program Area Manager
Health Emergency Intelligence, Surveillance and Laboratory Programme
WHO Emergency Preparedness and Response
WHO Regional Office for Africa
P O Box. 06 Cité du Djoué, Brazzaville, Congo

Email: afrooutbreak@who.int / afrgoafroshoc@who.int

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